



Response to HHS Health Sector AI RFI: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955-AA13)

To: Department of Health and Human Services (HHS)

From: Citrix Systems, Inc., a Cloud Software Group company

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Subject: Comment on RFI: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955-AA13)

I. Executive Summary: The Infrastructure for AI Readiness

While much of the healthcare industry's focus remains on the development of AI-powered applications, Citrix identifies the primary bottleneck to adoption as the deployment and delivery layer. In a healthcare environment characterized by shared workstations, mobile clinicians, and strict HIPAA requirements, an AI tool is only as effective as its availability at the point of care.

Citrix provides an industry leading platform that serves as a digital foundation for many healthcare organizations in support of delivering secure, connected healthcare services. With a footprint that includes a majority of the largest healthcare organizations in the United States and over 4,900 healthcare customers worldwide, our role is to provide the governance layer and platform that allows healthcare organizations to transition from AI experimentation to large-scale clinical use without compromising security or productivity. Our platform also helps remove friction at the bedside to ensure that technology works invisibly in the background, allowing care teams to stay focused on patients.

II. Response to Specific RFI Questions (1,3,5,6)

Q1: What are the primary barriers to the adoption of AI in clinical settings?

Citrix identifies the primary barrier to AI adoption as a deployment gap — the technical and operational friction that prevents an AI insight from reaching a clinician at the point of care.

To overcome this, HHS should address the following structural barriers:

- **The "Last Mile" delivery problem:** High-performing AI models have low adoption in clinical settings because they are not integrated into the roaming workflows of a hospital. In environments dominated by shared workstations, a clinician cannot afford the "cognitive tax" of logging into a separate, disconnected AI application.



- **Compute and hardware disparity:** Modern clinical AI agents require significant computational power. Most healthcare systems are burdened by "technical debt," utilizing legacy endpoints that cannot natively support resource-heavy AI processing.
- **Provider abrasion and "Login Fatigue":** Clinicians are facing record levels of burnout. If an AI tool adds even 30 seconds to a workflow or requires additional authentication events, it will be bypassed.
- **Governance-Induced latency:** The traditional process for vetting and provisioning new software in a hospital takes months, preventing clinical teams from iterating on AI tools at the speed of innovation.

Q1 Recommendation:

HHS should prioritize frameworks that support unified workspace delivery. By decoupling AI applications from physical hardware constraints; this allows healthcare organizations to scale innovation instantly across legacy infrastructure while eliminating clinician "login fatigue" through seamless, governed workflow integration at the point of care.

Q3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist, and what role, if any, should HHS play to help address them?

The AI regulatory and legal framework is rapidly evolving, fragmented across US state regulations, as well as global regulations that have extraterritorial applicability such as the EU AI Act. Technology vendors provide the AI capabilities and are regulated as such, but the extent to how those AI capabilities are deployed and used, together with the associated legal and regulatory risks, at its core rests with the legal, security and compliance leadership teams within the individual healthcare organizations. Fundamentally, it is the type of tools and technologies that should be implemented that HHS can best help influence organizations to meet their regulatory, legal and customer obligations.

Given the pace of change, the guidance should be technology agnostic to accommodate the unknown. Insofar as the implementation aspect of AI is concerned, the primary challenge for non-medical device AI is the "Shadow AI" security gap and subsequent data drift, where clinicians may input Protected Health Information (PHI) into unauthorized public AI models. This not only leads to sensitive data residing on unmanaged endpoints but also creates the risk that these public models could learn



from and retain PHI, potentially compromising patient privacy. To address this, robust controls must be implemented to prevent the use of public AI models within the healthcare workplace, ensuring that only approved, governed AI tools are utilized for clinical purposes.

To address these risks, Citrix recommends that HHS prioritize a Zero Trust Access framework that allows Risk & Resilience Leaders (CISOs) to quantify risk reduction in an objective manner that will support their compliance and financial objectives, including objective and verifiable data points that are required by the insurance market to underwrite the associated risks.

- **Granular, Role-Based Governance for AI Identities:** HHS should encourage adaptive, role based, and context aware access policies where AI availability and scope of permitted use is tied strictly to a user's clinical role and context aware data loss prevention (DLP) rules. Since 71% of CISOs report that AI has access to core systems, but only 16% govern that access effectively, Citrix recommends a governed environment that enforces least-privilege for AI "identities" to prevent unauthorized data exfiltration together with DLP rules that limit activities such as printing, pasting or downloading of sensitive data.
- **Improving MTTR and MTTC Metrics:** Underwriters now demand proof of strict controls to offer competitive coverage. Citrix suggests the use of virtualized, governed sessions that enable IT to:
 - **Reduce Mean Time to Contain (MTTC):** Quarantine unstable or compromised devices within 30 minutes of threat detection.
 - **Improve Mean Time to Respond (MTTR):** Slashing response times from hours to minutes using automated session isolation and lockout for risky behavior.
- **Standardized Patching Cycles:** To lower premiums and justify deductibles, organizations must prove cybersecurity maturity through patching cycle time. By centralizing AI application delivery, IT leaders can automate policy updates, ensuring that the clinical ecosystem is hardened against attack precursors without manual deskside toil.
- **Quantifiable Risk Posture:** Citrix recommends that HHS incentivize the use of **Agentic AI Posture Scores** and **Cyber Risk Quantification (CRQ)**. By containing AI processing within hosted sessions, hospitals can demonstrate a measurable reduction in "probable loss" (e.g., from \$5M to \$1M) by preventing PHI from ever residing on local, unmanaged endpoints.



- **Unified Forensic Auditability:** Evaluating liability is a fact specific analysis and requires a "provable" audit trail. Citrix recommends that HHS prioritize policies supporting **unified telemetry** that tracks AI interactions across all devices. This allows compliance teams to cut audit preparation time from **10 days to 2 days**, providing board-ready reporting that supports indemnification and insurance negotiations.

Q3 Recommendation:

HHS should prioritize the adoption of a **Zero Trust Access framework** that enforces context and role-based governance and unified telemetry for clinical AI. By delivering AI tools through a governed, virtualized environment rather than on local endpoints, healthcare organizations can effectively minimize the "Shadow AI" security gap, prevent PHI data drift, and improve quantifiable risk metrics—such as Mean Time to Contain (MTTC)—which are essential for maintaining cyber-insurance eligibility and organizational resilience.

Q5. What type of workforce, skills, and resources (e.g., educational and training, financial, etc.) are needed to enable and support the adoption and use of AI in clinical care? How can HHS support these needs?

The successful adoption of clinical AI requires a workforce shift from traditional "break-fix" IT to **Clinical AI-Ops**. Currently, healthcare IT teams are overburdened with manual system maintenance, particularly on securing traditional PC endpoints, leaving little capacity for the specialized oversight and optimization that AI requires. Citrix recommends the following workforce and resource priorities:

- **Transitioning IT to AI orchestrators:** The workforce needs a new class of professionals who understand both clinical workflow and the delivery of high-compute AI models.
- **Reclaiming workforce capacity through automation:** IT staff cannot develop new AI skills while buried under high ticket volumes and manual patching.
- **Continuous enablement via telemetry:** Training should not be a one-time event, but a continuous process driven by real-world usage data.

Q5 Recommendation:

HHS should leverage existing high-standard technical training infrastructures to establish a National Clinical AI-Ops Certification, while simultaneously incentivizing the use of operational excellence automation to reclaim the IT workforce capacity necessary to support and orchestrate AI innovation at scale.



Q6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations, and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

While many AI tools exhibit high accuracy in controlled environments, they often fall short in clinical practice due to high abrasion and the inability to maintain performance across a hospital's disparate hardware.

To ensure AI meets both performance and cost expectations, Citrix recommends that HHS prioritize metrics centered on workflow integration rather than just algorithmic accuracy.

- **Addressing performance shortfalls through "Time-to-Chart":** AI performance in clinical care should be measured by its impact on the clinician's entire session, not just the tool's isolated response time.
- **Novel AI potential:** The greatest potential for cost reduction and outcome improvement lies in AI that acts as an invisible background assistant rather than a standalone application.
- **Cost-effectiveness via infrastructure optimization:** Many AI tools fail cost expectations because they may require expensive local hardware upgrades.

Q6 Recommendation: HHS should define AI performance and cost-effectiveness through the lens of Clinical Throughput; by prioritizing novel AI that is decoupled from hardware and integrated into roaming workspaces, HHS can ensure that innovation reduces provider burden and lowers healthcare costs without requiring costly infrastructure overhauls.

III. Proposed strategic framework for HHS

To successfully transition clinical AI from isolated pilots to a national standard, Citrix recommends that HHS prioritize a regulatory and reimbursement framework built on three interdependent pillars: *User Experience*, *Tailored Security*, and *Operational Excellence*. This framework is essential because even the most advanced AI model will fail if it is not accessible within the high-pressure, mobile environment of a hospital.

User Experience must be the primary driver of adoption; AI insights must be integrated into a roaming workspace that ensures sub-10-second access at the bedside, preventing the "login fatigue" that contributes to clinician burnout.

Tailored Security must provide the necessary guardrails through role-based access and governed sessions, effectively closing the "Shadow AI" gap and preventing data drift without introducing workflow friction.



Finally, *Operational Excellence* serves as the engine for workforce transformation. By automating the "manual toil" of infrastructure management such as patching and image delivery, healthcare systems can reclaim the IT capacity required to orchestrate and scale AI innovation. Together, these pillars form the Digital Foundation necessary for a resilient, AI-native healthcare ecosystem.

IV. Conclusion: Advancing secure, connected care

Citrix is committed to providing the digital foundation required to support a modern, AI-native healthcare system while protecting and enabling the data it needs to serve our communities. Our mission of secure, connected care focuses on reducing the fatigue and frustration of clinical teams by ensuring that technology operates invisibly and reliably in the background.

We believe that the success of accelerating AI in healthcare depends not only on the accuracy of clinical models but on a resilient and scalable delivery architecture that protects patient privacy while giving time back to the clinician. We recommend that HHS partner with industry leaders to define the national standards for Clinical AI-Ops, prioritizing speed, security, and stability as essential for the next generation of American clinical care.

V. Next Steps and Expert Consultation

Citrix welcomes the opportunity to further discuss these recommendations and share additional technical evidence regarding the orchestration of clinical AI at scale.

We invite HHS leadership to engage in a specialized briefing with our Healthcare Strategic Advisors and subject matter experts to explore how the Citrix platform can support the department's national objectives.

To schedule a follow-up discussion or a technical deep dive into these frameworks, please contact our Healthcare Strategy team at sarah.fink@citrix.com

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